Fillable practice sheets

Blank versions of the differential sheets. Each page keeps the row and column headings; you fill in the rest.

How to use them: **1.** Fill a page from memory, without notes. **2.** Check it against the study sheet page listed at the top and correct in another color. **3.** Repeat the pages you got wrong a few days later. Where a page says “Draw it,” sketch the timeline before you check.

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The mental status exam

Define each term in your own words, then describe what you would see. **Check your answers: study sheets p. 1.**

Thought process and content

Term	What it sounds like
Linear, goal-directed	
Circumstantial	
Tangential	
Flight of ideas	
Derailment	
Perseveration	
Clanging	
Neologisms; word salad	
Ruminations	
Delusions (name 5 types)	
Capgras · Cotard · Fregoli	

Speech and movement

	Depression	Mania	Anxiety
Speech			
Psychomotor			

Movement finding	Points to
Waxy flexibility, echolalia, negativism	
Cogwheeling, shuffling	
Lip smacking, grimacing	
Torticollis, oculogyric crisis	

Perception and dissociation

Term	Definition
Illusion vs hallucination	
Hypnagogic; hypnopompic	
Depersonalization	
Derealization	
Fugue; jamais vu; déjà vu	

The one question that separates circumstantial from tangential: _____

Bipolar I, bipolar II, cyclothymia and depression

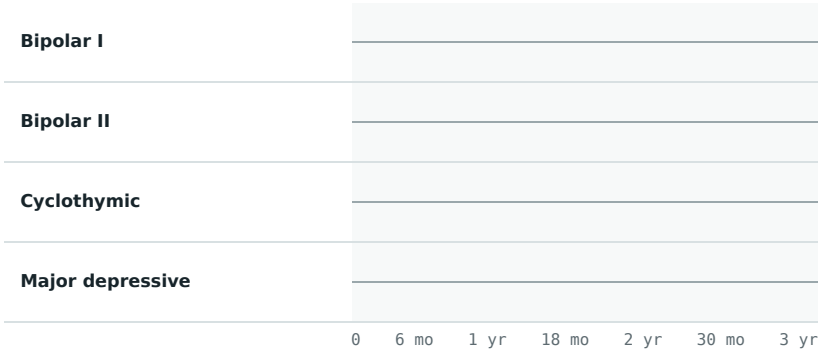
Sort by the most severe high the patient has ever had. **Check your answers: study sheets pp. 2–3.**

Diagnosis	Episodes required	Episodes that rule it out	Typical course	First-line treatment
Bipolar I disorder				
Bipolar II disorder				
Cyclothymic disorder				
Major depressive disorder				

Mania vs hypomania

	Mania	Hypomania
Duration		
Symptoms		
Severity		
Diagnosis it points to		

Draw it: mood chart (above the line = high, below = low)



Minimum duration to label a manic episode

Minimum duration for hypomania

What makes any high “mania” regardless of length?

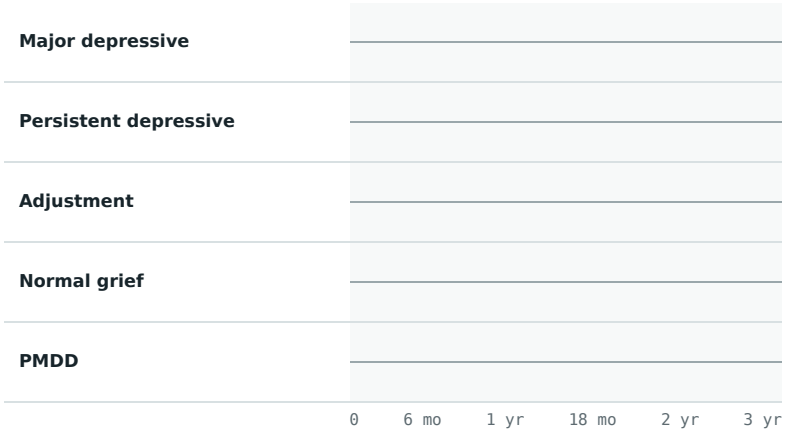
Ask about the lifetime history of highs, then: _____

Telling the depressive disorders apart

Fill in timing, presentation and the one feature that separates each. **Check your answers: study sheets pp. 4–5.**

Diagnosis	Timing	What it looks like	Key discriminator	First-line treatment
Major depressive disorder				
Persistent depressive disorder				
Adjustment disorder				
Normal grief				
Prolonged grief disorder				
Premenstrual dysphoric disorder				
Substance- or medical-induced				

Draw it: timeline



GRIEF OR DEPRESSION?

Symptoms common in normal grief

GRIEF OR DEPRESSION?

Features that point to major depression

Two questions that sort most low-mood presentations: _____

The antidepressants

Name the drugs in each class, how they work, when to choose them and what to watch for. **Check your answers: study sheets p. 6.**

Class	Drugs	Mechanism	Best used for	Watch for
SSRI				
SNRI				
NDRI				
α2 antagonist				
SARI				
Tricyclic (TCA)				
MAOI				

Match the need to the drug

Neuropathic pain → _____

Sexual side effects on an SSRI → _____

Insomnia and weight loss → _____

Smoking cessation → _____

Seizure disorder: avoid → _____

High overdose risk: avoid → _____

Persistent priapism after a sleep aid → _____

Rising blood pressure on an SNRI → _____

Psychotic depression → _____

Two failed trials: options → _____

EMERGENCY

Serotonin syndrome: cause, signs, treatment

Before any antidepressant, always ask about: _____

Mood stabilizers

Mark acute mania and maintenance, then fill in mechanism and adverse effects. **Check your answers: study sheets p. 7.**

Drug	Acute mania?	Maintenance?	Mechanism	Common adverse effects	Rare but serious
Lithium					
Valproic acid					
Carbamazepine					
Lamotrigine					
Second-generation antipsychotics					

Lithium toxicity

Level	Signs
Mild	
Moderate	
Severe	

Diagnosis	First-line medication
Bipolar I disorder	
Bipolar II disorder	
Cyclothymic disorder	

CLINICAL PEARL

Mania that starts after an antidepressant: what is the diagnosis and what do you do?

BOARD FAVORITE

One adverse effect for each mood stabilizer

"Is this acute mania or prevention?" Acute → Maintenance → _____

Telling the anxiety disorders apart

Separate them by what the fear is about and what the person does. **Check your answers: study sheets p. 8.**

Disorder	What the fear is about	What the person does	Duration	First-line treatment	Tell it from
Generalized anxiety disorder					
Panic disorder					
Agoraphobia					
Social anxiety disorder					
Specific phobia					
Separation anxiety disorder					

A SYMPTOM
Panic attack

A DISORDER
Panic disorder

EXPECTED ATTACKS
Attacks with a trigger point to…

Ask the patient: _____

OCD and its relatives

Name the thought, name the behavior, and ask how the person feels about it. **Check your answers: study sheets p. 9.**

Disorder	The thought or urge	The repetitive behavior	Insight and attitude	First-line treatment
Obsessive-compulsive disorder				
Body dysmorphic disorder				
Hoarding disorder				
Trichotillomania				
Excoriation disorder				
FOR COMPARISON				
Obsessive-compulsive personality disorder				
Illness anxiety disorder				

GAD
Worry

OCD
Obsession

PSYCHOSIS
Delusion

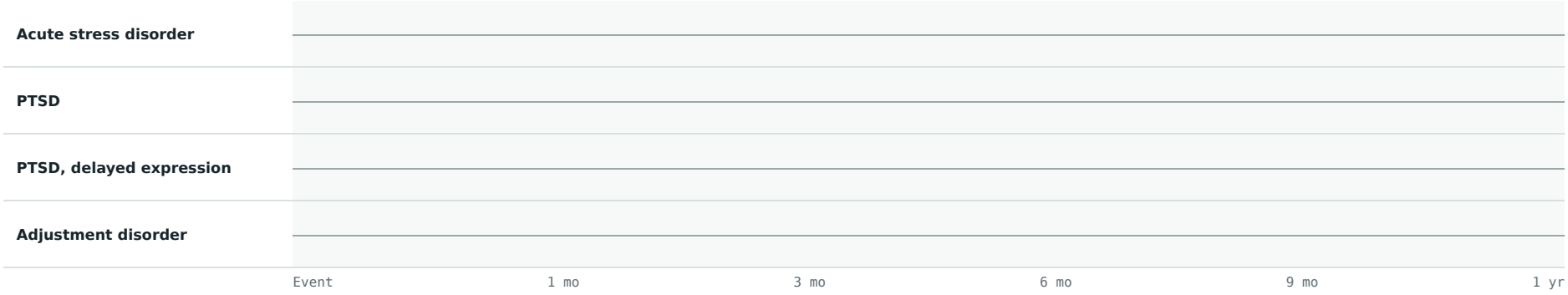
Ask: _____

Acute stress, PTSD and adjustment disorder

Was it a trauma or an ordinary stressor, and how long has it been? **Check your answers: study sheets p. 10.**

Diagnosis	What happened	Timing	Core symptoms	First-line treatment
Acute stress disorder				
Posttraumatic stress disorder				
Adjustment disorder				
Prolonged grief disorder				

Draw it: timeline after the event



PTSD cluster 1 (how many needed?) _____ PTSD cluster 2 _____

PTSD cluster 3 _____ PTSD cluster 4 _____

A divorce or job loss points to: _____

Medications for anxiety, OCD and trauma

Daily drug that prevents symptoms vs as-needed drug for the moment. **Check your answers: study sheets p. 11.**

Disorder	Daily, long-term medication	As needed or add-on	Avoid or not helpful
Generalized anxiety disorder			
Panic disorder			
Agoraphobia			
Social anxiety disorder			
Specific phobia			
PTSD			
OCD			
BDD; hoarding			
Trichotillomania, excoriation			

Drug	Mechanism	Use and key effects
Buspirone		
Benzodiazepines		
Hydroxyzine		
Propranolol		
Prazosin		

“Every day or for the moment?” Every day → _____

Telling the “schizo” disorders apart

Is there frank psychosis, how long has it lasted, and where do mood episodes fall? **Check your answers: study sheets p. 12.**

Disorder	Frank psychosis?	Duration	Mood episodes	Function and social pattern	Tell-tale sign
PERSONALITY DISORDERS (CLUSTER A)					
Schizoid personality disorder					
Schizotypal personality disorder					
PSYCHOTIC DISORDERS					
Brief psychotic disorder					
Schizophreniform disorder					
Schizophrenia					
Schizoaffective disorder					
Delusional disorder					

Under ___ → brief psychotic _____ ___ to ___ → schizophreniform _____ ___ or more → schizophrenia _____

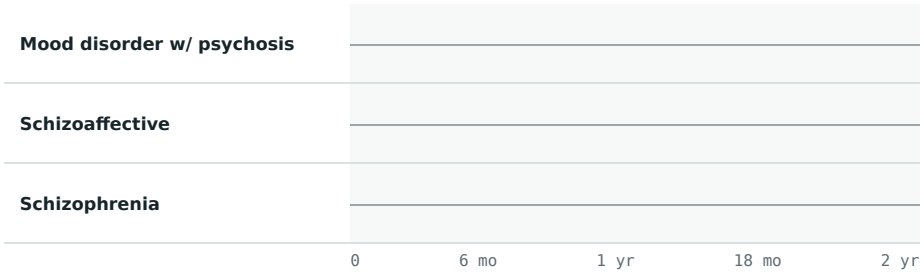
Symptoms that count toward schizophrenia (at least one must be…): _____

Psychosis and mood; primary or secondary?

Draw the timeline, then rule out substances, medications, delirium and medical illness. **Check your answers: study sheets pp. 13–14.**

Diagnosis	Psychosis during mood episodes?	Psychosis without a mood episode?	Mood episodes for most of the illness?
Mood disorder with psychotic features			
Schizoaffective disorder			
Schizophrenia			

Draw it: mood episodes and psychosis



Cause	Onset	Attention	Hallucinations	Other clues
Schizophrenia spectrum				
Mood disorder with psychosis				
Substance or medication				
Medical condition				
Delirium				
Dementia with Lewy bodies				

Red flag 1 _____ Red flag 2 _____

Red flag 3 _____ Red flag 4 _____

“When the mood was normal, was the patient still psychotic?” Never → _____

Antipsychotics

Group the drugs, then map each dopamine pathway to its effect. **Check your answers: study sheets p. 15.**

Group	Mechanism	Examples	Side-effect tilt
Typical, high potency			
Typical, low potency			
Atypical: D2 + 5-HT2A			
D2 partial agonists			
Muscarinic (Cobenfy)			

Pathway	Runs from → to	Blocking D2 here causes
Mesolimbic		
Mesocortical		
Nigrostriatal		
Tuberoinfundibular		

Worst for weight and diabetes _____

Worst for EPS and prolactin _____

Worst for akathisia _____

Agranulocytosis _____

Acute dystonia: treat with _____

Long-acting injections _____

	Neuroleptic malignant syndrome	Serotonin syndrome
Cause		
Onset		
Findings		
Treatment		

“Which side effects can this patient least afford?” _____

Schizophrenia: course, prognosis and side effects over time

Fill in the prognosis table and place each side effect on the timeline. **Check your answers: study sheets p. 16.**

Factor	Good prognosis	Poor prognosis
Sex		
Onset		
Main symptoms		
Mood symptoms		
Premorbid functioning		
Social support		
Relapses		
Other		

Positive symptoms (pathway) _____

Negative symptoms (pathway) _____

Suicide: die / attempt _____

Side effect	When	What it looks like	Response
Acute dystonia			
Acute parkinsonism			
Akathisia			
Neuroleptic malignant syndrome			
Metabolic syndrome			
Tardive dyskinesia			

“How did it start, and what was the person like before?” _____

The ten personality disorders

Name the cluster, then the core pattern that ties the traits together. **Check your answers: study sheets p. 17.**

Disorder	Core pattern	Look for	Easily confused with
CLUSTER A ·			
Paranoid			
Schizoid			
Schizotypal			
CLUSTER B ·			
Antisocial			
Borderline			
Histrionic			
Narcissistic			
CLUSTER C ·			
Avoidant			
Dependent			
Obsessive-compulsive (OCPD)			

Cluster A is _____, B is _____, C is _____. All personality disorders start by: _____

Look-alikes and treatment

Lifelong pattern vs episode, and the person's attitude toward the behavior. **Check your answers: study sheets pp. 18–19.**

Personality disorder	Look-alike	Points to the personality disorder	Points to the look-alike
Borderline			
Avoidant			
Schizoid			
Schizotypal			
Paranoid			
OCPD			
Antisocial			
Narcissistic			
Dependent			
Histrionic			

THERAPY FIRST

Therapies with evidence for borderline personality disorder

MEDICATIONS

What the evidence says, and target symptoms

Ask: _____

Delirium and catatonia

Compare delirium with its look-alikes, list causes, and plan prevention. **Check your answers: study sheets p. 20.**

	Delirium	Dementia	Depression
Onset			
Course			
Attention, arousal			
Other features			

Do	Don't

A _____ E _____

I _____ O _____

U _____ T _____

I _____ P _____

S _____ S _____

Deliriogenic drug classes (5) _____

CATATONIA
Signs

CATATONIA
Causes

CATATONIA
Treatment

Hypoactive delirium looks like _____

How to test attention _____

Any sudden change over a day or two: think _____ first. Ask: _____

SUBSTANCE USE · PRACTICE

Intoxication and withdrawal

Fill in mechanism, intoxication, withdrawal and what makes each dangerous. **Check your answers: study sheets p. 21.**

Substance	Mechanism	Intoxication	Withdrawal	Danger
Alcohol				
Benzodiazepines				
Opioids				
Cocaine, methamphetamine				
Cannabis				
MDMA (ecstasy)				
PCP				
Synthetic drugs				
Nicotine				

Which withdrawals can kill? _____

SUBSTANCE USE · TREATMENT · PRACTICE

Treating substance use disorders

Alcohol withdrawal and relapse prevention, opioid overdose and maintenance, nicotine. **Check your answers: study sheets p. 22.**

Draw it: withdrawal symptoms, seizures, delirium tremens



ED management _____

When to admit _____

CIWA cut-offs _____

Who gets scheduled benzodiazepines _____

Drug	Mechanism	Best for	Watch for
Naltrexone			
Acamprosate			
Disulfiram			

Drug	Mechanism	Key points
Naloxone		
Methadone		
Buprenorphine		
Naltrexone (opioids)		
Varenicline		
Bupropion		
Nicotine replacement		

Stimulant use disorder: best-evidence treatment _____

Opioid use disorder in pregnancy _____

Before buprenorphine, ask: _____

Telling the psychotherapies apart

Name each therapy by what happens in the room and how it is set up. **Check your answers: study sheets p. 23.**

Therapy	Core idea	What happens in session	Format and length	Best for
Supportive				
Psychodynamic / psychoanalytic				
Cognitive behavioral (CBT)				
Behavioral activation				
Dialectical behavior (DBT)				
Motivational interviewing				
Interpersonal (IPT)				

Therapy first line	Both are common	Medication first line

“What does the therapist actually do in session, and how often, for how long?” _____

PSYCHOTHERAPY · PRACTICE

Matching the therapy to the problem

Name the specific therapy and what it involves. **Check your answers: study sheets p. 24.**

Problem	Therapy	What it involves
Panic disorder		
GAD		
Agoraphobia		
Specific phobia		
OCD		
PTSD		
Trichotillomania, excoriation		
Hoarding; body dysmorphic		
Depression		
Borderline personality disorder		
Psychosis		
Ambivalence about a habit		

Supportive intervention	Purpose	Example phrase
Reflecting		
Naming		
Normalizing		
Reframing		

PATIENT → CLINICIAN
Transference

CLINICIAN → PATIENT
Countertransference

“What exactly does the patient need to face, or to change?” _____